



Fiona Stanley Fremantle Hospitals Group

Goals of patient care

Reference #: FSFH-HW-POL-0053

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Hospital wide	Medical, Nursing, Midwifery and Allied Health

1. Introduction

This document provides clinicians with directions for the correct implementation of the Goals of Patient Care (GoPC) process including completion of the GoPC documentation.

GoPC have a particular focus on the clinical care of patients with advanced, life limiting conditions. It requires clinicians to have open and honest discussions with patients and their families within 48 hours of admission to hospital, so clear ceilings of care and end of life wishes can be established.

If the patient does not have capacity to determine their GoPC then refer to the Treatment Hierarchy of Treatment Decision-Makers ([Appendix 1](#)).

2. Terminology

Goals of Patient Care (GoPC)	Process which prompts and facilitates proactive shared decision making between the clinicians and patients and / or person responsible / family / carer(s) to ensure treatment provided is aligned to the patient's preferences, needs, values, wishes, cultural obligations and beliefs or practices. It establishes and documents the most medically appropriate, agreed GoPC that will apply in the event of the patient's clinical presentation and/or deterioration.
GoPC Summary /GoPC eForm	A state-wide care planning electronic form (eForm) used to aid clinicians in the Goals of Patient Care process and provide a summary of the shared discussion. At FSFHG sites the form is accessed from the GoPC eForm filter in the digital medical record (DMR) . Submitted

	(completed) forms are saved behind the “Not for CPR” filter (refer to Appendix 4 for diagrams of these filters) The GoPC eForm is transferrable to other hospital facilities if the patient is transferred during that episode of care. A photocopy of the GoPC eForm is to be included in transfer paperwork.
Supportive and Palliative Care Tool (SPICT™)	Tool used to help identify people at risk of deteriorating and/or dying with one or multiple advance conditions. This enables holistic, palliative care needs assessment and care planning. (See Appendix 2).
Advance Care Plan (ACP)	A record of advance care planning discussions. It can be used to document a wide range of decisions and details important to the individual, to assist carers and health professionals with planning and delivery of care. ACP’s are not underpinned by legislation.
Enduring Power of Guardianship (EPG)	A written document where a person nominates another person to make personal, lifestyle and healthcare treatment decisions on their behalf if they are unable to make reasonable judgments about these matters in the future. Treatment decisions contained within the EPG can only be enacted if they are clinically appropriate The scope of decisions can be limited by the appointer and should be specified in the EPG form
Advance Health Directive (AHD)	For the purposes of this document, AHD refers to a document recognised under the Guardianship and Administration Act 1990 which records a competent adult’s decisions about possible future treatment. Treatment decisions recorded in a valid AHD must be followed in circumstances where the maker of the AHD can no longer make or communicate the decision themselves. Refer to the FSFHG Advance Health Directive and Enduring Power of Guardianship policy and procedure.
Junior Medical Officer (JMO)	For the purposes of this document a JMO is an intern or resident medical Officer (RMOs)
Senior Medical Officer (SMO)	For the purposes of this document, this term refers to a Registrar or Consultant Medical Officer

See [Appendix 3](#) for a complete glossary of terms

3. Policy

A senior medical officer is required to complete the GoPC eForm for patients who meet the inclusion criteria (section 4.1) within 48 hours of admission to hospital or earlier.

If clinical deterioration is likely; urgent interventions are planned; or for patients with advanced, life limiting conditions the GoPC eForm should be completed at the earliest opportunity.

All senior clinicians who have a significant role in any stage of the patient journey are to consider completing a GoPC eForm.

In the absence of a completed GoPC eForm, the default will be full resuscitation, unless senior clinicians have deemed resuscitation measures inappropriate for the patient's best interests.

All healthcare professionals (internal and external) will comply with the agreed GoPC until such time as the GoPC is reviewed and renegotiated with the patient / person responsible.

3.1. Medical Officer Responsibilities

3.1.1. The Senior Medical Officer (SMO) is responsible for:

- listening and responding to the patient, family/carers questions
- initiating timely discussions around treatment options, treatment limiting orders and non-beneficial treatments
- enabling the patient / person responsible to make informed GoPC decisions.
- accurately reflecting the patient's wishes, values and preferences in the GoPC eForm.
- Confirming completed GoPC form with patient and/or carer
- Print for end of bed paperwork

Any Junior Medical Officer (JMO) may complete Section 1 of the GoPC eForm.

The discussion with the patient in Section 2, 3 and 4 of the GoPC eForm must be held by a SMO (Registrar or Consultant) and documented at the earliest available opportunity.

A JMO may complete all sections of the eForm under the guidance and instruction of the SMO following or during their discussion with the patient / family /carer(s).

The Registrar or Consultant's details must be captured on the eForm as a record of persons involved in the discussion.

Medical staff are to include GoPC discussions in discharge summaries.

3.1.2. Members of the medical team are responsible for ensuring they are aware of the patient's treatment preferences as outlined in the GoPC eForm and complying with them at all times within the current admission or time period for which the eForm is valid

- The GoPC eForm is valid for the current admission or on transfer to another site if there is no clinical change that may require the eForm to be revised and a new one completed.
- A Consultant can endorse the GoPC form for extended use beyond this admission for up to 12 months.

3.1.3. If the patient's condition or preference changes necessitating a new GoPC plan, then a new eForm is to be completed by the relevant medical staff as per 3.1.1 above:

- document reasons for the change to the GoPC eForm in the DMR
- Previous GoPC in end of bed paperwork must be discarded and replaced with most recent GoPC.

3.2. Multidisciplinary team member responsibilities

- Nursing and Allied Health staff and other members of the multidisciplinary team (MDT) are responsible for contributing to the process and providing input into the decisions outlined in the eForm as necessary.
- MDT members are encouraged to initiate discussion about completing the GoPC process with medical members of the treating team if GoPC have not been considered.

4. Procedure

4.1. Inclusion criteria

- Patients with AHD/ACP with health-related instructions
- Patients with advanced, life limiting conditions
- Patients that meet the general indicators for deteriorating health outlined in the Supportive & Palliative Care Indicators Tool (SPICT™). Refer to [Appendix 2](#)
- Patients that meet the Supportive & Palliative Care Indicators Tool (SPICT™) criteria ([Appendix 2](#))

- Patients over the age of 70 years
- Other patients as determined by the treating Clinician.
- For patients incapable of making an informed decision for GoPC, the discussion is to be held with their family / carer / person responsible, according to the [Appendix 1](#) South Metropolitan Health Service (SMHS) Treatment Hierarchy.

4.2. Exclusion criteria

- Nil.

4.3. Initial FSFHG presentation

- The Medical Officer reviews the DMR for the GoPC via the “Not for CPR” filter on the left side of the front page of the DMR.
- Where a GoPC is not in place, consider the inclusion criteria and commence documentation as appropriate.
 - For Aboriginal patients it is recommended that the assistance of an Aboriginal Liaison Officer be enlisted to ensure that goals are formulated in accordance with the patient’s cultural values.
- Include use of interpreter service as appropriate.
- For patients with an identified AHD or EPG, refer to the [FSFHG Advance Health Directives and Enduring Power of Guardianship policy and procedure](#) and for discussions about end of life care choices refer to the Comprehensive Care HUB page, (Comprehensive Care (health.wa.gov.au))
 - If the AHD is no longer representing the agreed treatment management plan during the patient’s hospital admission, the AHD can be revoked. The GoPC eForm is completed to reflect the amended treatment plan for the current admission.
 - If the patient is discharged, they are encouraged to create a new AHD at their earliest convenience.

4.4. Subsequent FSFHG presentations

- The Medical Officer reviews the presence or absence of a GoPC, confirms its intent with the patient and / or person responsible and revises and/or extends GoPC eForm as appropriate.

There is no prompt in the DMR to indicate that a current GoPC form exists. Staff must check for the current form behind the “CPR?” filter.

A Clinical Alert will be noted on the front page of the patient’s DMR, if the patient has an AHD correctly recorded as a Clinical Alert.

- Information regarding the conversation and the GoPC eForm must be recorded in the DMR

4.5. Clinical handover

- It is the responsibility of all clinical staff involved in the handover of patient care at any stage of the patient's journey to include information on the details of the GoPC in all handovers including handovers to transport staff, staff from other WA Health sites and staff from external private organisations.
- Nursing staff are to document evidence of the GoPC eForm on the Clinical Handover Sheet.
- The patient's GoPC status will be recorded on patient journey boards or whiteboards where available and will identify GoPCs where treatment responses have been limited e.g. Not for CPR (NFR), Not for MET (NFM) response.
- The General Practitioner (GP) may receive a copy of the GoPC eForm upon request to the Medical team

4.6. Discharge and transfer

- Prior to discharge or transfer of the patient to another facility, the eForm is to be printed and kept with the patient's medical records for transfer.
 - If the patient is being discharged home give the printed eForm to the patient.
- If a patient is admitted/transferred to FSFHG sites with a completed and valid GoPC form completed at another site, medical staff are responsible for the completion of the GoPC eform at FSFHG.

5. Compliance/Performance Monitoring

- The Comprehensive Care Committee will be responsible for the evaluation of the GoPC program and compliance with this policy. An annual report will be completed and submitted to the Committee

6. Related Policy Documents

Department of Health, WA:

- [WA Health Consent to Treatment Policy](#)

FSFHG:

- Advance Health Directives, Advance Care Plans and Enduring Power of Guardianship (FSFH-HW-POL-0031)

7. Other related documents/resources:

- [WA Advance Health Directives](#)
- [WA Health Advance Care Planning](#)
- [WA Health Advance Health Directives in Practice](#)
- [Supportive and Palliative Indicators Tool](#)
- [Enduring Power of Guardianship \(EPG\)](#)
- [W.A. Office of Public Advocate](#)

8. Related Standards

NSQHS Standards:

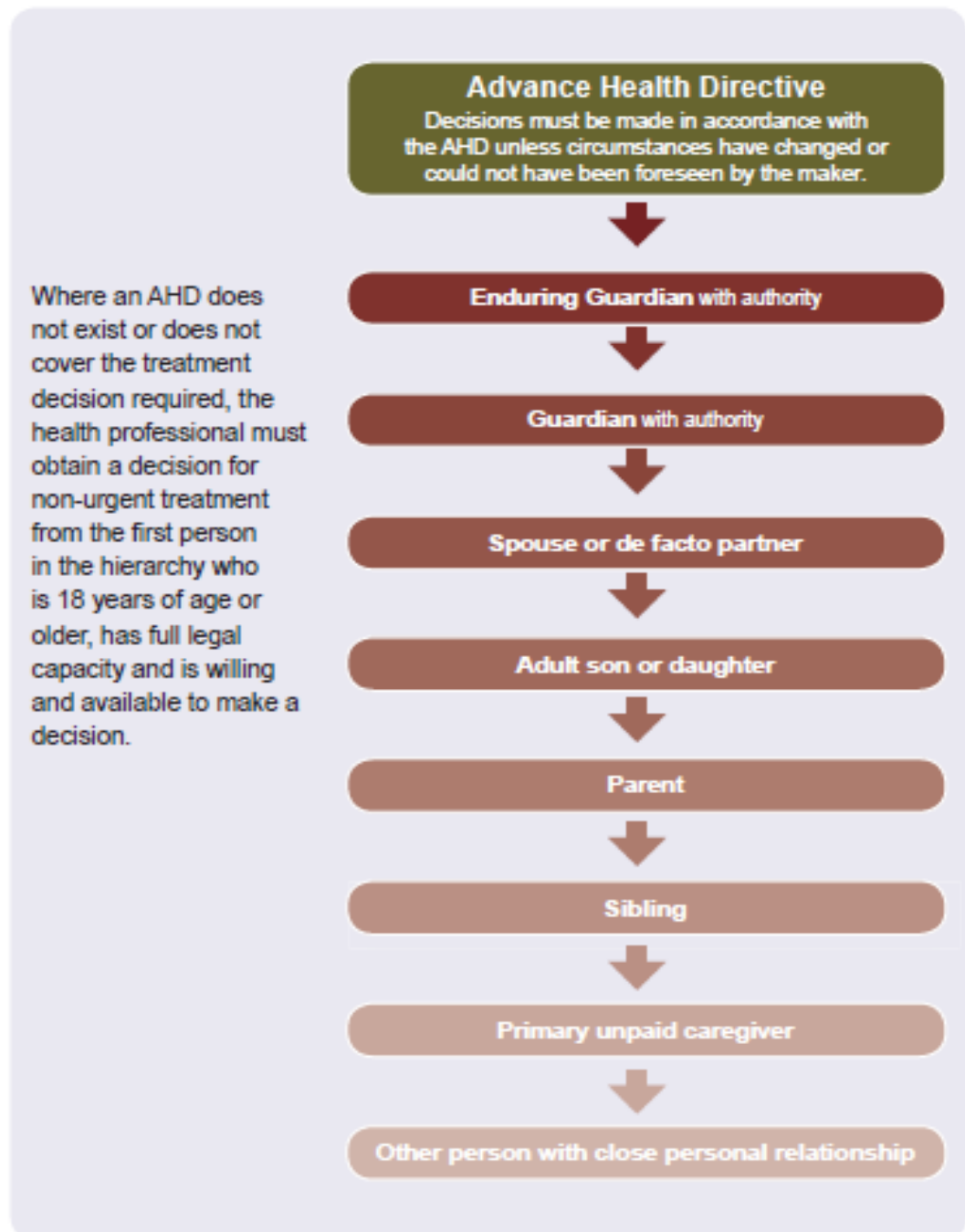
- Clinical Governance
- Partnering with Consumers
- Comprehensive Care
- Communicating for Safety
- Recognising and Responding to Acute Deterioration

9. Authorisation

EXECUTIVE SPONSOR: Director of Clinical Services					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	10/2019	Innovation Consultant, SMHS	Recognising and Responding to Acute Deterioration Committee	FSFHG Policy Committee	10/2022
2	06/2023	Strategic program manager	Comp Care Committee, Clinical Governance Committee, End of Life Care Comm	FSFHG Policy and Pracgoatice Committee	06/2027

10. Appendices

10.1. Appendix 1: Hierarchy of Treatment Decision- Makers



Government of Western Australia: Office of the Public Advocate 2018

10.2. Appendix 2: SPICT™ Tool & guidance for use



Supportive and Palliative Care Indicators Tool (SPICT™)



The SPICT™ is used to help identify people whose health is deteriorating. Assess for unmet supportive and palliative care needs. Plan care.

Look for any general indicators of poor or deteriorating health.

- Unplanned hospital admission(s).
- Performance status is poor or deteriorating, with limited reversibility. (Person stays in bed or in a chair for more than half the day.)
- Depends on others for care due to increasing physical and/or mental health problems. The person's carer needs more help and support.
- Progressive weight loss; remains underweight; low muscle mass.
- Persistent symptoms despite optimal treatment of underlying condition(s).
- The person (or family) asks for palliative care; chooses to reduce, stop or not have treatment; or wishes to focus on quality of life.

Look for clinical indicators of one or multiple life-limiting conditions.

Cancer

- Too frail for cancer treatment or treatment is for symptom control.
- Functional ability is deteriorating due to progressive cancer.

Dementia/ frailty

- Unable to dress, walk or eat without help.
- Eating and drinking less; difficulty with swallowing.
- Urinary and faecal incontinence.
- Not able to communicate by speaking; little social interaction.
- Frequent falls; fractured femur.
- Recurrent febrile episodes or infections; aspiration pneumonia.

Neurological disease

- Progressive deterioration in physical and/or cognitive function despite optimal therapy.
- Speech problems with increasing difficulty communicating and/or progressive difficulty with swallowing.
- Recurrent aspiration pneumonia; breathless or respiratory failure.
- Persistent paralysis after stroke with significant loss of function and ongoing disability.

Heart / vascular disease

- Heart failure or extensive, untreatable coronary artery disease; with breathlessness or chest pain at rest or on minimal effort.
- Severe, inoperable peripheral vascular disease.

Respiratory disease

- Severe, chronic lung disease; with breathlessness at rest or on minimal effort between exacerbations.
- Persistent hypoxia needing long term oxygen therapy.
- Has needed ventilation for respiratory failure or ventilation is contraindicated.

Other conditions

- Deteriorating with other conditions, multiple conditions and/or complications that are not reversible; best available treatment has a poor outcome.

Kidney disease

- Stage 4 or 5 chronic kidney disease (eGFR < 30ml/min) with deteriorating health.
- Kidney failure complicating other life limiting conditions or treatments.
- Stopping or not starting dialysis.

Liver disease

- Cirrhosis with one or more complications in the past year:
 - diuretic resistant ascites
 - hepatic encephalopathy
 - hepatorenal syndrome
 - bacterial peritonitis
 - recurrent variceal bleeds
- Liver transplant is not possible.

Review current care and care planning.

- Review current treatment and medication to make sure the person receives optimal care; minimise polypharmacy.
- Consider referral for specialist assessment if symptoms or problems are complex and difficult to manage.
- Agree a current and future care plan with the person and their family/people close to them. Support carers.
- Plan ahead early if loss of decision-making capacity is likely.
- Record, share, and review care plans.



Why use the SPiCT™?

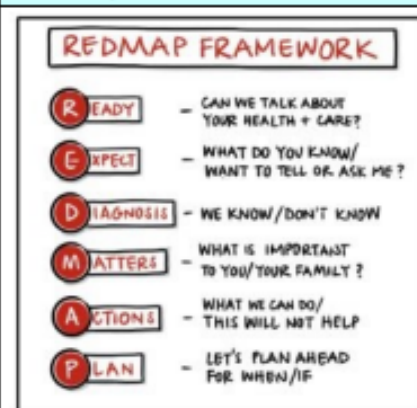
SPiCT™ helps clinicians identify people with one or more general indicators of poor or deteriorating health and clinical signs of life-limiting conditions for assessment and care planning.

SPiCT™ looks for changes in health status, burden of illness and increasing care needs. Integrate a holistic palliative care approach with best available treatment of underlying illnesses. Timely identification avoids harm and improves treatment and care of patients and families.

Using SPiCT™ to assess people's needs and plan care.

- **Unplanned hospital admission**, more clinic **visits** or a **decline in health status**: review current care, treatment and medication; discuss future options; plan for managing further deterioration.
- **Poorly controlled symptoms**: review and optimise available treatment of underlying conditions; stop medicines/interventions/tests not of benefit; use effective palliative symptom control measures.
- People who are **increasingly dependent on others** due to deteriorating functional ability, physical frailty and/or mental health problems often need additional care and support.
- **Complex symptoms** or other patient/family **needs**; consider specialist palliative care review or involve another appropriate specialist or service.
- Assess **decision-making capacity**. Plan ahead if this will deteriorate. Record details of close family/friends, Power of Attorney or legal proxies. Involve in decision-making if capacity is impaired.
- Identify people who need proactive, **coordinated care in the community** from the primary care team and/or other community staff and services. Involve the local community. Support carers.
- Agree, record, share, and plan to review advance/anticipatory care plans (**ACP**); include plans for urgent/ emergency care and treatment if the person's health deteriorates or care at home changes.

Talking about future care planning



■ Talk about:

- Benefits, harms and costs of hospital admission, outpatient visits, tests and treatments (e.g. IV antibiotics/fluids; surgery; cancer treatments, interventions for heart or kidney disease; tube feeding; oxygen/ventilation).
- Treatments that will not work or have a poor outcome for this person. (eg. cardiopulmonary resuscitation)
- Choosing legal proxy decision-makers in case the person's decision-making capacity is lost in the future.
- What a person would like; anything they do not want.
- Help and support for family/ informal carers.

Tips on starting conversations about deteriorating health

- *I wish we had a treatment for... Could we talk about what **we can do** if that's not possible?*
- *I am glad you feel better and I **hope** you will stay well, but I **am worried** that you could get ill again...*
- *Can we talk about how we manage **not knowing exactly** what will happen and when?*
- *If you got less well in the future, what would **be important** for you? What **would she say** about this?*
- *Some people want to talk about whether to go to hospital or be at home if they are seriously ill....*

10.3. Appendix 3: Glossary of Terms

Adult	In this policy the term Adult refers to a person 18 years and over to whom the <i>Guardianship and Administration Act 1990</i> applies.
Advance Care Directive (ACD)	Advance Care Directive is a collective term for Advance Health Directives, Enduring Powers of Guardianship and Common Law Directives
Advance Care Plan (ACP)	An advance care plan is a record of advance care planning discussions. It can be used to document a wide range of decisions which are important. It can assist carers and health professionals to understand what is important to the individual when they plan and deliver care. Advance Care Plans are not underpinned by legislation. 'My Advance Care Plan' is a tool developed by the Government of Western Australia. Department of Health Advance care plans are a tool designed to fill in additional detail to assist carers and health professionals with planning and delivering care.
Advance Health Directive (AHD)	In this policy, an AHD means a document recognised under the <i>Guardianship and Administration Act 1990</i> ⁽⁴⁾ which records a competent adult's decisions about possible future treatment. Treatment decisions recorded in a valid AHD must be followed in circumstances where the maker of the AHD can no longer make or communicate the decision themselves.
Capacity	In the context of medical treatment, a person has full legal capacity if he/she is capable of understanding the nature, purpose and consequences of the proposed treatment. Capacity must always be assessed in the context of the decision that is to be made. The <i>Mental Health Act 2014</i> (s15) ⁽³⁰⁾ defines a person as having capacity when they: ▪ Understand any information or advice about the decision that is required ▪ Understand the matters involved in the decision ▪ Understand the effect of the decision ▪ Weigh up the above factors for the purpose of making the treatment decision ▪ Communicate the decision in some way. Under the <i>Mental Health Act 2014</i>, adults are presumed to have capacity unless shown not to. A child is presumed NOT to have capacity about a decision unless he or she is shown to have that capacity.
Carer	The <i>Carers Recognition Act 2004</i> , Section 5 ⁽³²⁾ , defines a carer as a person who provides ongoing care or assistance to another person who has a disability, a chronic illness, including a mental illness, or a person because of frailty, requires assistance with carrying out everyday tasks ⁽⁷⁾ . The <i>Carers Recognition Act 2004</i> , Section 5 ⁽³²⁾ , also clarifies who is NOT a carer.
Common law	Law derived from decisions made by judges in courts or tribunals in respect of particular cases, to be contrasted with statute law made by parliaments; also known as 'case law'; may set precedent to be followed.

Common Law Directive (CLD)	'Common law directives' (CLD) are written or verbal communications which convey a person's preferences regarding health treatment to be provided or withheld in specified future circumstances. There are no formal requirements in relation to common law directives. However, there can be considerable difficulties in establishing that a particular CLD is legally valid and can be followed. For this reason they are not recommended. Under the Guardianship & Administration Act, common law directives are valid.
Competent patient	A patient who has capacity
Consent (to medical treatment)	In the context of healthcare, consent to treatment is a person's agreement that a health professional can proceed to perform a specific proposed treatment
Enduring Guardian	A person appointed under an Enduring Power of Guardianship to make personal, lifestyle and treatment decisions on behalf of the adult person who makes the appointment
Enduring Power of Guardianship (EPG)	In this policy, an EPG means a document recognised under the Guardianship and Administration Act 1990 ⁽⁴⁾ in which a person nominates a competent adult to be an Enduring Guardian (see the Guardianship and Administration Act 1990 Part 9A)
Escalation protocol	The protocol that sets out the organisational response required for different levels of abnormal physiological measurements or other observed deterioration. The protocol applies to care of all patients at all times
Goals of Patient Care (GOPC)	Goals of Patient Care (GOPC) is a clinical process used to establish the most medically appropriate, realistic and agreed goal of patient care that will apply in the event of the patient's clinical deterioration. GOPC prompts and facilitates proactive shared discussion and decision making between clinicians and patients and/or person responsible/family/carer(s) to ensure treatment provided is aligned to the patient's preferences, individual needs and values. The GOPC aims to improve the delivery of care during clinical deterioration, congruent with patient opinion through a common understanding of the goal of care and individual escalation plan. It also aims to reduce patient exposure to unwanted/non beneficial and burdensome treatments.
Goals of Patient Care (GoPC) Summary	The GOPC Summary is a care planning eForm used to aide clinicians in the Goals of Patient Care process and to provide a summary of the shared discussion. It is used to identify, document and communicate the most medically appropriate, agreed goal of patient care that will apply in the event of the patient's clinical deterioration. The agreed goal provides detail of the patient's individual escalation plan including any limitations to treatment for the current admission or during planned extended use (beyond the admission).
Guardian	An adult appointed by the State Administrative Tribunal to make wide-ranging decisions, including treatment decisions, on behalf of an adult person who does not have the capacity to make such decisions for him/herself (see the <i>Guardianship and Administration Act 1990</i> Part 5).
Hierarchy of decision-makers for treatment	The hierarchy of treatment decision makers for treatment relates to the consent process which must be for patients under legal incapacity who are unable to make reasonable judgments about treatment. The hierarchy identifies the criteria for each person responsible and the order in which they should be consulted about a treatment decision (refer to <i>Guardianship and Administration Act 1990</i>). For a diagrammatic representation

	of 'Hierarchy of decision-makers for treatment', refer to the <i>WA Health Consent to Treatment Policy</i>
Living Will	The term 'living will' is an expression sometimes used to describe a record in which a person communicates their views regarding their anticipated future healthcare decisions, such as whether the person consents or withholds consent in relation to specified treatment decisions which they anticipate will arise in the future. Living wills are intended to come into effect when their maker can no longer make and communicate decisions about their healthcare. Living wills may include: • AHDs which are formally recognised as legal and binding upon persons responsible for care
Life-limiting illness/condition	A disease, condition or injury that is likely to result in death, but not restricted to the terminal stage when death is imminent
Mature minor	A person under the age of 18 who has sufficient emotional and intellectual capacity to fully comprehend the nature, consequences and risks of a proposed action (for example, a treatment decision or a decision to release health information), irrespective of whether a parent/substitute decision maker consents to it. A child who is assessed to be a mature minor in relation to a particular treatment decision may either give or refuse consent to it
Non-beneficial treatments	Interventions that will not be effective in treating a patient's medical condition or improving their quality of life. Non-beneficial treatment may include interventions such as diagnostic tests, medications, artificial hydration and nutrition, intensive care, and medical or surgical procedures. Non-beneficial treatment is sometimes referred to as futile treatment, but this is not a preferred term.
Person responsible	Under the <i>Guardianship and Administration Act 1990</i>, a person who may legitimately make a treatment decision on behalf of a patient who is unable to make reasonable judgments for him/herself. Refer to The hierarchy of treatment decision-makers for treatment to determine who the 'person responsible' is.
State Administrative Tribunal (SAT)	Independent tribunal established under the <i>State Administrative Tribunal Act 2004</i> ⁽³⁹⁾ to hear disputes and make determinations about a range of administrative matters. e.g. proceedings relevant to health include health practitioners' disciplinary matters, guardianship matters, including disputes regarding the Advance Health Directive, Enduring Power of Guardianship, Enduring Power of Attorney and Common Law Directive.
Substitute Decision Maker (SDM)	A general term for a person who is either appointed or identified to make care decisions on behalf of a person whose decision making capability is impaired. A substitute decision maker (SDM) is sometimes called the 'person responsible'. Depending on the situation, a substitute decision maker may be: • Formally appointed by the patient under the <i>Guardianship and Administration Act 1990</i> ⁽⁴⁾, in which case they would be referred to as the Enduring Guardian; or

	Formally appointed by the State Administrative Tribunal under the <i>Guardianship and Administration Act 1990</i>⁽⁴⁾, in which case they would be referred to as the Guardian; or The person responsible according to the treatment hierarchy 'hierarchy of decision makers' identified in the <i>Guardianship and Administration Act 1990</i>
Treatment	Any medical, surgical or dental treatment or other health care, including a life-sustaining measure or palliative care.
Treatment decision	A decision to consent or refuse consent to the commencement or continuation of any treatment of the person.
Treatment-Limiting Orders	Orders, treatment ceilings, instructions or decisions that involve the reduction, withdrawal or withholding of life-sustaining treatment. These may include 'no cardiopulmonary resuscitation' or 'not for resuscitation' and other treatments considered non-beneficial.

10.4. Appendix 4: BOSSnet DMR Filter Diagrams

Figure1: Goals of Patient Care eForm filter (access blank forms here)

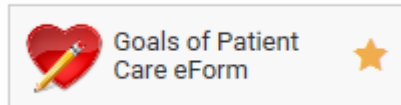


Figure 2: Not for CPR filter (completed GoPC forms saved here)

